Lisinopril
Product Information

BRAND / TRADE NAMES

Zestoretic | Prinzide

HOW SUPPLIED

Lisinopril and hydrochlorothiazide 10-12.5 tablets: white to off-white, round tablets. Available in bottles of 30, 90, and 100 tablets.

STORAGE & HANDLING

Store at 20-25 degrees C (68-77 degrees F). Excursions permitted. Keep tightly closed. Protect from moisture.

KEY DRUG FACTS

Generic Name: lisinopril and hydrochlorothiazide
Drug Class: ACE inhibitors, plain
Manufacturer: AstraZeneca Pharmaceuticals LP
Route: Oral
Rx Status: Prescription Only

QR CODE

For healthcare professionals only. Refer to full prescribing information before clinical use. Data sourced from US FDA OpenFDA.

Route Oral

Rx Only

Form Tablet

lisinopril and hydrochlorothiazide

Lisinopril

ACE inhibitors, plain

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Route	Class	Rx Only
Oral	ACE inhibitors, plain	Yes

Mfd by AstraZeneca Pharmaceuticals LP

PRESCRIBING INFORMATION BROCHURE

Clinical Information

INDICATIONS & USAGE

Lisinopril and hydrochlorothiazide tablets are indicated for the treatment of hypertension, to lower blood pressure. Lowering blood pressure reduces the risk of fatal and non-fatal cardiovascular events, primarily strokes and myocardial infarctions.

CONTRAINDICATIONS

Contraindicated in patients who are hypersensitive to any component of this product and in patients with a history of angioedema related to previous treatment with an angiotensin converting enzyme inhibitor and in patients with hereditary or idiopathic angioedema.

CLINICAL USE NOTES

- Monitor blood pressure at each visit and after dose changes.
- Counsel patients on signs of angioedema (swelling of face/throat).
- Avoid use in pregnancy — teratogenic risk in 2nd and 3rd trimesters.
- Monitor serum electrolytes and renal function regularly.
- Counsel patients to report any new or worsening symptoms promptly.

PATIENT POPULATION

Adults: 18 years and older
Geriatric: Use with caution; monitor closely
Pregnancy: Contraindicated — teratogenic risk
Pediatric: Safety not established — consult PI

Drug Overview

DESCRIPTION

Lisinopril and hydrochlorothiazide tablets combine an angiotensin converting enzyme inhibitor, lisinopril, and a diuretic, hydrochlorothiazide. Lisinopril is an oral long-acting angiotensin converting enzyme inhibitor. It is a white to off-white crystalline powder.

DOSAGE & ADMINISTRATION

Lisinopril monotherapy is effective in once-daily doses of 10 mg to 80 mg, while hydrochlorothiazide monotherapy is effective in doses of 12.5 mg to 50 mg per day. The antihypertensive response rates generally increased with increasing dose of either component.

DOSAGE QUICK REFERENCE

Starting dose: 10 mg once daily
Max dose: 80 mg per day
Frequency: Once daily
Administration: Take with or without food
Titration: Gradual dose escalation recommended

MECHANISM OF ACTION

- Inhibits angiotensin-converting enzyme (ACE), reducing angiotensin II formation.
- Decreases vasoconstriction and aldosterone secretion.
- Lowers peripheral vascular resistance and blood pressure.
- Reduces cardiac preload and afterload.

Safety Information

WARNINGS & PRECAUTIONS

Anaphylactoid and Possibly Related Reactions: Patients receiving ACE inhibitors may be subject to a variety of adverse reactions, some of them serious. Angioedema of the face, extremities, lips, tongue, glottis, and larynx has been reported in patients treated with ACE inhibitors.

ADVERSE REACTIONS

In clinical trials with lisinopril and hydrochlorothiazide tablets no adverse experiences peculiar to this combination drug have been observed. The most common adverse reactions include dizziness, headache, cough, fatigue, and orthostatic hypotension.

MONITORING PARAMETERS

- Blood pressure — at each visit and after dose changes
- Renal function (eGFR) — before and periodically during therapy
- Serum electrolytes (K+, Na+) — regularly
- Signs of angioedema (swelling of face/throat) — ongoing
- Signs of hypersensitivity or serious adverse reactions — ongoing

NOTABLE DRUG INTERACTIONS

NSAIDs: May reduce antihypertensive effect
Potassium-sparing diuretics: Risk of hyperkalemia
Lithium: Increased lithium toxicity risk
Digoxin: Electrolyte imbalance may increase toxicity